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[CAUSE TITLE — to be prepared by the advocate]

Before the District Consumer Disputes Redressal Commission, Chandigarh

Consumer Complaint No. DC/44/CC/423/2026

WRITTEN ARGUMENTS ON BEHALF OF THE COMPLAINANTS

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References: "WS" is the written statement of the Opposite Party (PGIMER) dated 26.06.2026, filed on 01.07.2026; "OP Annexure n" is an annexure to the WS, cited by the page number of the WS bundle; "Annexure C-n" is an annexure to the complaint, cited by the page number of the complaint paper-book.

I. BRIEF FACTS

1. The complainants are the legal heirs of Late Sh. Manohar ("the patient"), who died at the Opposite Party's hospital on 13.03.2026 (Annexure C-11).

2. On 10.03.2026 the patient was at the Government Multi Specialty Hospital, Sector 16, Chandigarh, where his haemoglobin was recorded as 8.2 g/dL (Annexure C-7, page 47). At 12:14 am on 11.03.2026 that hospital referred him "to PGI Emergency regarding his recent ECG findings [CAD ?ACS?] (Wellens)" (Annexure C-6, page 46).

3. At the Opposite Party's emergency the patient was registered at 2:04 am on 11.03.2026 in the "GENERAL" category (Annexure C-8, page 51). The emergency note records "Imp - Wellens Syndrome" and the plan "Shift to CCU (ACC)" (Annexure C-8, page 52).

4. The patient was admitted to the Opposite Party's cardiac care unit (CCU) at 10:10 am on 11.03.2026, on payment of an advance of Rs. 2,000 (OP Annexure 10, page 23). The Opposite Party's CCU sheet records "retrosternal chest pain × 1 day", an ECG rate of 123 with new T-wave changes in leads V2–V4 (OP Annexure 2, page 14), the diagnosis "CAD – USA (Wellen's pattern)" and "anaemia", and the plan "correct anaemia" (OP Annexure 3, page 15). Its own haemogram records haemoglobin of 7.5 g/dL against a normal range of 13.5–17.5 (OP Annexure 6, page 18).

5. The Opposite Party treated the patient for acute coronary syndrome (WS para 17) and admits that it "owed a duty of care to the deceased" (WS para 20-G).

6. On 12.03.2026 a request for one unit of blood was received at the Opposite Party's blood storage centre at 9:15 am; the unit was cross-matched and "kept ready for issue around 12 p.m." (OP Annexure 7, page 19; Annexure C-10, page 56). A sum of Rs. 150 was paid for cross-matching at 4:47 pm (Annexure C-10, page 58). The Opposite Party admits that the blood "was never released or issued" (WS para 8).

7. On 12.03.2026 the Opposite Party moved the patient out of the CCU. It admits "a temporary delay in the availability of a ward bed" and that the patient was kept "on the trolley" (WS paras 10, 11, 20-b). By its own account he had been in the CCU for "nearly 31 hours" from 10:10 am on 11.03.2026 (WS preliminary objection VI), which places the shift at about 5:10 pm on 12.03.2026.

8. At about 9:00 pm on 12.03.2026 the patient was brought to the CCU. The Opposite Party says that this happened "only once", that his complaint was "mild breathlessness", that his pulse and saturation were checked and that he was auscultated (WS para 14). He was not re-admitted. The complainants' case is that he had chest pain (complaint paras 12–14).

9. The last entry on the Opposite Party's CCU vital chart is at 9:00 pm on 12.03.2026: blood pressure 90/60, pulse 82 (OP Annexure 5, page 17). The next entry on the same chart is the nurse's note that the patient was "Received in CCU at 12:15", "unresponsive & pulse was not palpable". He was declared dead at 12:51 am on 13.03.2026 (OP Annexure 5; WS para 18).

10. The Opposite Party's own medical certificate of cause of death records, in Part I, (a) Type I respiratory failure, (b) aspiration

pneumonia and (c) coronary artery disease, and in Part II pulmonary tuberculosis (Annexure C-11; OP Annexure 9, page 22).

II. PROCEDURAL HISTORY

11. The complaint was filed on 07.04.2026. By order dated 22.04.2026 it was admitted and notice issued "with the directions to OP(s) to file written statement within 30 days from the date of receipt of notice extendable for 15 days (total 45 days)".

12. On 29.05.2026 the Opposite Party appeared through counsel and was directed to file its written version "alongwith evidence". The written version, dated 26.06.2026, was filed on 01.07.2026; the order of that date records that evidence had not been filed.

13. By order dated 07.08.2026 this Commission recorded: "Evidence by OP not filed despite being provided sufficient opportunity. Hence the opportunity to file evidence by OP No.1 is closed by order of this Commission."

14. By order dated 14.10.2026 the complainants' opportunity to file a rejoinder was closed, the Opposite Party's written arguments were taken on record, and the complainants were permitted to file written arguments within three days. These written arguments therefore also answer the written statement, as no rejoinder is on record. [Filing date and any application for delay: to be completed by the advocate.]

III. POINTS FOR DETERMINATION

15. It is respectfully submitted that the following points arise:

a. Whether the complainants are consumers and the complaint is within limitation and jurisdiction.

b. Whether the Opposite Party, having itself diagnosed "CAD – USA (Wellen's pattern)" and anaemia, rendered the care it had undertaken: the planned correction of anaemia, observation in the CCU, and assessment when the patient was brought back at about 9:00 pm.

c. Whether the patient was adequately observed before he was received unresponsive at 12:15 am on 13.03.2026.

d. Whether the deficiencies caused or contributed to the death.

e. To what relief the complainants are entitled.

IV. SUBMISSIONS

A. The complaint is maintainable

16. The Opposite Party charged an advance of Rs. 2,000 for the very admission in question (OP Annexure 10, page 23) and Rs. 150 for cross-matching (Annexure C-10, page 58). Nowhere in its written statement does it dispute that the complainants are consumers or that the service was rendered for consideration. Paid medical service is "service" within the Consumer Protection Act (Indian Medical Association v. V.P. Shantha, (1995) 6 SCC 651) [advocate to check the citation and its current status against the official report].

17. The Opposite Party expressly admits the date of the cause of action (13.03.2026), limitation and territorial jurisdiction (WS paras 22–23). Its objection in para 2 rests only on its contention that no deficiency exists, which is a question of merits.

B. The diagnosis recorded by the Opposite Party and the duty it admits

18. The Opposite Party's own records describe the patient's condition without qualification: "Imp - Wellens Syndrome" (Annexure C-8, page 52) and "CAD – USA (Wellen's pattern)" with "anaemia" (OP Annexure 3, page 15). It treated him for acute coronary syndrome (WS para 17) and admits that it owed him a duty of care (WS para 20-G).

19. The written statement says that the referring hospital's ECG showed "only '?Wellens Syndrome' the '?' indicating that the diagnosis was merely suspected" (WS para 3). With respect, the referral reads "[CAD ?ACS?] (Wellens)": the question marks qualify "ACS", while " (Wellens)" is written on the next line, underlined, without a question mark (Annexure C-6, page 46). The Opposite Party's own emergency and CCU records then record Wellens without any query.

20. The complainants do not rest their case on heart failure. The Opposite Party's own tests show a negative troponin and a normal ejection fraction (OP Annexures 1, 2). The case is that a patient diagnosed by the Opposite Party itself with unstable coronary disease and anaemia required the care the Opposite Party itself planned.

C. Blood was planned, cross-matched, kept ready and never issued

21. The Opposite Party's plan was to "correct anaemia" (OP Annexure 3, page 15), and its own haemogram showed haemoglobin of 7.5 g/dL (OP Annexure 6, page 18). One unit was cross-matched at 12:55 pm and

"kept ready for issue" (Annexure C-10, page 56; OP Annexure 7, page 19). The Opposite Party admits that it was "never released or issued" (WS para 8). No transfusion order appears in its drug chart (OP Annexure 8, page 21).

22. The defence that the blood was "only a precautionary standby measure" for haemoptysis and that "No medical indication for routine transfusion existed" (WS preliminary objection V) does not match the Opposite Party's own written plan to "correct anaemia". Its blood bank records that the unit was kept ready "for the patient" (OP Annexure 7).

23. The Opposite Party pleads that blood was not to be transfused unless there was "a further decline in the hemoglobin levels" (WS para 15). No haemoglobin test after the value of 7.5 g/dL appears in the record placed before this Commission.

24. The Opposite Party's own blood reaction form prints the instruction "Get the Component issued only when the need for transfusion arises" (Annexure C-10, page 56). The decision that the need had arisen rested with its treating doctors, whose CCU duty doctor, on the Opposite Party's own case, told the family that there was "no immediate medical urgency for blood transfusion" (WS para 15). The letter of 01.06.2026 stating that "no attendant came to collect" the unit (OP Annexure 7) was written after the complaint, at the request of the Opposite Party's Department of Cardiology dated 29.05.2026, and cannot displace the Opposite Party's own plan and admissions.

25. The Opposite Party relies on a threshold of 7 g/dL "per established guidelines" (WS paras 3, 8, 15, 20-D) but has filed no guideline. Whether transfusion was indicated in a patient being treated for acute coronary syndrome, and with what urgency, is a matter on which [expert opinion / medical literature — to be completed by the advocate].

D. The patient was moved out of the CCU onto a trolley

26. The Opposite Party admits that "there was a temporary delay in the availability of a ward bed due to PGIMER's extremely heavy patient load" and that the patient was kept "on the trolley" (WS para 11). The complainants do not fault the Opposite Party for the shortage of beds. The complaint is about the care given to the patient while he waited.

27. The Opposite Party asserts that "Even on the trolley, the patient continued to be under the monitoring and care of PGIMER's medical team, as conclusively evidenced by the vital chart maintained by the nursing staff" (WS para 11). The vital chart it has annexed contains no

entry after 9:00 pm on 12.03.2026 until the patient was received unresponsive at 12:15 am (OP Annexure 5, page 17).

28. The Opposite Party further pleads that transfers between the ward and the CCU are "coordinated between the Ward Senior Resident and the CCU Senior Resident, with the patient accompanied by hospital attendant or medical staff" (WS para 12), and that the patient was shifted "only after adequate monitoring and clinical assessment" (WS para 20-b). No transfer note, assessment or record of such coordination has been produced. At 3:00 pm on 12.03.2026, about two hours before the shift, its chart records a pulse of 114 and blood pressure of about 93/60 (OP Annexure 5).

E. The night of 12/13 March 2026: no observation is recorded for 3 hours 15 minutes

29. On the Opposite Party's own papers: the patient was on a trolley outside the CCU; at about 9:00 pm he was brought to the CCU once, examined as stated in WS para 14, and not re-admitted; the last recorded observation is at 9:00 pm (blood pressure 90/60, against 110/73 at 1:00 pm); and the next entry is that he was received at 12:15 am, unresponsive and without a palpable pulse (OP Annexure 5, page 17).

30. The Opposite Party's account of the 9:00 pm examination is given "to the best of our knowledge" (WS para 14). It states that saturation was "within normal limits", but its CCU chart has no column for oxygen saturation and no saturation value for that examination appears in any document produced. It states that the findings were "similar to those at initial admission" with "no tachycardia or tachypnoea" (WS para 12), whereas its own chart for the day of admission records pulse rates of 125 to 128 and respiratory rates of 24 to 26 (OP Annexure 5). The written statement does not refer to the blood pressure of 90/60 recorded at 9:00 pm. No electrocardiogram of that examination is on record; the only ECG mentioned in the night note is one at 12:51 am, at the declaration of death.

31. It is therefore submitted that a patient whom the Opposite Party had itself diagnosed with "CAD – USA (Wellen's pattern)" remained on a trolley for 3 hours and 15 minutes with no observation recorded, and reached the CCU without a pulse.

F. The CCU chart does not support "monitored constantly"

32. The written statement pleads that the patient's vitals were "monitored constantly" (preliminary objection VI) and that he showed "hemodynamic stability throughout" (WS paras 15, 20-E). The CCU chart contains ten recorded readings over about 35 hours, with intervals of 4 hours (9:00 pm to 1:00 am), 8 hours (1:00 am to 9:00 am), 4 hours (9:00 am to 1:00 pm) and 6 hours (3:00 pm to 9:00 pm), and records blood pressure of 90/49 (6:00 pm on 11.03.2026), 87/46 (9:00 am on 12.03.2026) and 90/60 (9:00 pm on 12.03.2026) (OP Annexure 5, page 17). Whether these values called for escalation is a matter for [expert opinion — to be completed].

G. Defences without documents; evidence closed; records not produced

33. The Opposite Party's opportunity to lead evidence stands closed (order dated 07.08.2026). Its written statement is verified "to the best of my knowledge and belief" by its authorised representative; no letter of authority is annexed (WS page 12).

34. The Opposite Party relies on the comments of its Department of Cardiology to a Medical Grievances Committee (preliminary objection II), on an "existing protocol at PGIMER" (WS paras 6, 10, 12), on guidelines for a haemoglobin threshold (WS paras 3, 8, 15, 20-D), on counselling of the family (preliminary objection IV; para 20-a), and on findings at intubation during resuscitation (WS para 19). None of these documents has been produced. The written statement is also silent on prayers (iv) and (v) of the complaint, which seek the details of the duty doctors and the preservation of the records and CCTV footage of 12–13.03.2026.

35. It is submitted that the Opposite Party's own documents may be read against it, and that its averments on the matters set out in the preceding paragraph rest on the written statement alone. [Legal effect of the closure of evidence and authorities: to be completed by the advocate.]

H. The cause of death certified by the Opposite Party

36. The Opposite Party pleads that the death was "attributable entirely to the cumulative and progressive deterioration of these underlying conditions" (preliminary objection IV). Its own certificate lists coronary artery disease in Part I of the cause of death (Annexure C-11; OP Annexure 9). The word "entirely" does not match the Opposite Party's own certificate.

37. The Opposite Party bases "aspiration" on "clinical findings observed during endotracheal intubation at the time of attempted resuscitation" (WS para 19). No resuscitation or intubation record has been produced. At about 9:00 pm, on the Opposite Party's own case, the chest findings were "similar to when he was admitted" (WS para 14); its emergency note on admission records "Chest – B/L AE +" (Annexure C-8, page 52). Whether aspiration pneumonia existed before the arrest, or followed it, is a matter for [expert opinion — to be completed].

I. Causation

38. The sequence of events is established by the Opposite Party's own records. Whether closer observation, an electrocardiogram at 9:00 pm, or the correction of anaemia that the Opposite Party itself planned would have altered the outcome is a matter on which [expert opinion / submissions — to be completed by the advocate].

39. The Opposite Party pleads that coronary angiography was "deferred" on clinical grounds and that the family was informed (WS para 20-a). No note of the deferral or of any counselling has been produced. The deferral, as pleaded, underlines the need for close observation of a patient whose coronary disease remained untreated by angiography.

V. REPLY TO THE PRELIMINARY OBJECTIONS

40. **Preliminary objection I** (no deficiency; "guideline directed" care): denied. The Opposite Party's own plan to correct anaemia was not carried out; the patient was moved onto a trolley; no observation is recorded from 9:00 pm to 12:15 am; no guideline has been filed (Parts C to F above).

41. **Preliminary objection II** (suppression of facts; committee comments): denied. The comments relied upon have not been produced; they are the views of the treating department; no evidence was led.

42. **Preliminary objection III** (severity "grossly exaggerates"; NT-proBNP; BNP 37.4; normal LVEF): the complainants rely on the Opposite Party's own diagnosis of "CAD – USA (Wellen's pattern)" and anaemia, not on heart failure. The point-of-care print relied on for BNP 37.4 bears no name or date and also flags myoglobin 176 and D-dimer 3280 as abnormal (OP Annexure 1, page 13).

43. **Preliminary objection IV** (co-morbidities; death "entirely" due to them; counselling): the Opposite Party itself admits co-existing

coronary artery disease and certified it in the cause of death; no counselling note is produced (Part H).

44. **Preliminary objection V** (blood as a standby; no indication): denied for the reasons in Part C.

45. **Preliminary objection VI** ("nearly 31 hours"; "monitored constantly"; ECG repeated): the time is accepted and fixes the shift at about 5:10 pm; "monitored constantly" is answered in Part F; the ECG copies annexed (OP Annexures 11, 12) bear no legible date, time or name.

46. **Preliminary objection VII** (quantum): see Part VIII.

VI. REPLY TO THE WRITTEN STATEMENT ON MERITS

47. As the opportunity to file a rejoinder was closed, the complainants answer the material paragraphs here; every averment not admitted below is denied.

- **Para 3:** the referral is answered in para 19 above.
- **Paras 6 and 7:** it is accepted that Annexure C-9 is the Opposite Party's emergency chart (WS para 7). The CCU chart (OP Annexure 5) is answered in Parts E and F.
- **Para 8:** the admission that the blood was "never released or issued" is relied upon (Part C).
- **Paras 10 to 12:** the shift and the trolley are admitted; the remaining averments are answered in Parts D and E.
- **Paras 14 and 15:** the 9:00 pm visit and the "no immediate medical urgency" statement are admitted by the Opposite Party; the rest is answered in Parts C and E.
- **Para 17:** treatment for acute coronary syndrome is admitted and relied upon.
- **Para 18:** the times of 12:15 am and 12:51 am are accepted (see Part VII).
- **Para 19:** answered in Part H.
- **Para 20 (a) to (j):** the admission of duty in 20-G is relied upon; the deferral in 20-a is answered in para 39; the remaining sub-paragraphs are denied.
- **Para 21:** answered in Part VIII.
- **Paras 22 and 23:** the admissions are relied upon.

48. [Reply to the Opposite Party's written arguments dated 14.10.2026: to be added after a copy is obtained from the registry.]

VII. CORRECTIONS TO THE COMPLAINT

49. In fairness to this Commission, the complainants state the following corrections openly:

a. Para 18 of the complaint reads the time of death on the certificate as 1:53 AM. The record shows 12:51 am, which the complainants accept. The time of about 12:10–12:15 am in para 18 is the time the family found the patient unresponsive, which matches the Opposite Party's note that he was received at 12:15 am.

b. Paras 16, 18 and 21 refer to the patient's widow as "Complainant No.2". As per the memo of parties she is Complainant No.3.

c. Para 7 and ground A state that no monitoring was conducted after 8:30 am on 11.03.2026. That statement was based on the emergency chart (Annexure C-9). The Opposite Party's CCU chart (OP Annexure 5) records later entries, and the complainants rely instead on the intervals and values in that chart (Part F).

d. Para 14 states that there was no blood pressure monitoring at night. The CCU chart records a blood pressure of 90/60 at 9:00 pm; the complainants' case is that nothing is recorded after that entry (Part E).

VIII. RELIEF

50. The complainants have claimed Rs. 60,00,000 with interest at 12% per annum, Rs. 2,50,000 towards medical expenses, Rs. 1,00,000 towards litigation expenses, the details of the duty doctors, and the preservation of records and CCTV footage. Payments to the Opposite Party documented on record are Rs. 2,000 (OP Annexure 10) and Rs. 150 (Annexure C-10). [Submissions on quantum, income and dependency, with the documents to be relied upon: to be completed by the advocate.]

51. The Opposite Party accepts Maharaja Agrasen Hospital v. Master Rishabh Sharma (C.A. 6619 of 2016, decided on 16.12.2019), Sarla Verma v. Delhi Transport Corporation, (2009) 6 SCC 121, and Charan Singh v. Healing Touch Hospital, (2000) 7 SCC 668, as "settled law governing compensation in medical negligence matters" (WS para 20-H) [advocate to check the citations against the official reports; the

complaint cites Charan Singh as "(2000) 7 SCC 68" and Sarla Verma as "III (2009) ACC 708 (SC)"].

52. The Opposite Party's prayer for exemplary costs is unfounded. The complaint is founded on the Opposite Party's own records, and the complainants confine their submissions to those records.

IX. DOCUMENTS RELIED UPON

Document	Description	Page
Annexure C-6	GMSH-16 referral summary, 10–11.03.2026	46
Annexure C-7	GMSH-16 blood tests and ECGs, 10.03.2026	47–50
Annexure C-8	PGIMER emergency record and treatment sheet	51–54
Annexure C-9	PGIMER emergency BP chart	55
Annexure C-10	Blood reaction form, identification slip, receipt	56–58
Annexure C-11	Medical certificate of cause of death	59 (index)
OP Annexures 1–3	Point-of-care print; CCU admission sheet	13–15
OP Annexure 5	CCU vital chart and nurse's note	17
OP Annexures 6–8	Haemogram; blood bank letter and register; drug chart	18–21
OP Annexures 9–12	Death certificate; admission record; ECG copies	22–25
Orders	22.04, 29.05, 01.07, 07.08 and 14.10.2026	—

[PRAYER, VERIFICATION, PLACE, DATE AND SIGNATURE — to be prepared by the advocate]